

Preliminary Qualitative Evidence and Field Observation on Medical Record Management in Government Hospital Clinics in Sri Lanka

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Submitted as supporting Evidence for the ClinicOne System Proposal

1. Introduction

This document presents preliminary qualitative evidence and field observations supporting the development of the proposed ClinicOne web-based clinical information management system. The purpose of this document is to provide contextual justification for the identified problem by documenting recurring challenges related to medical record management in Sri Lanka government hospital clinics.

Due to the limited availability of formally published academic research specifically addressing record management inefficiencies in local government hospital clinics, this document compiles insights derived from public discussions, personal observations, and primary survey findings.

2. Source of Observations

The observations presented in this document are derived from the following sources:

- Publicly available social media discussions within Sri Lankan online communities (e.g., Reddit forums related to healthcare experiences).
- Informal patient feedback and shared personal experiences.
- Direct observation of clinic administrative practices.
- Preliminary survey data collected from nursing staff through structured questionnaires.

These sources were reviewed to identify recurring patterns and common concerns regarding documentation practices and access to medical records.

3. Identified Recurring Issues

Based on qualitative observations gathered from public discussions (including LinkedIn, Reddit, and other social platforms), informal patient experiences, and preliminary survey insights, the following recurring challenges were identified in government hospital clinic settings:

3.1 Dependence on Paper-Based Record Systems

Government hospital clinics primarily rely on physical documentation, requiring patients to carry previous prescriptions, laboratory reports, and clinic notes for each visit. This system increases the risk of:

- Loss or damage of important documents
- Incomplete medical history during consultations
- Administrative inefficiencies in storing and retrieving files

The absence of centralized digital records may affect continuity of care, particularly for long-term or chronic patients.

3.2 Limited Direct Patient Access to Medical Records

It has been observed that hospital-maintained medical files are treated as institutional records, and patients may have restricted access to view, photograph, or obtain copies of their documentation. While record protection is essential for confidentiality, limited accessibility may reduce patient awareness of their own medical history and complicate long-term personal record keeping.

3.3 Requirement for Duplicate External Test Reports

When patients undergo diagnostic tests at private laboratories or external facilities, they are often required to provide multiple printed copies of their reports during government clinic visits. One copy is typically retained within hospital files. This reinforces dependency on physical documentation and increases logical responsibility for patients.

3.4 Consultation Appointment Delays and Scheduling Inefficiencies

Publicly shared discussions, including professional commentary on LinkedIn, highlight concerns regarding appointment delays in both public and private healthcare settings. Several contributing factors have been identified:

- Consultation in government hospitals often manage multiple responsibilities, including ward rounds, surgeries, emergency cases, academic teaching, and outpatient clinics. Overlapping duties may lead to cascading delays.
- Appointment system in certain settings may involve high patient volumes within limited time slots, making strict punctuality difficult to maintain.
- Delays early in the day (e.g., extended consultations or emergency interventions) can cause cumulative time shifts throughout the schedule.

These factors contribute to prolonged patient waiting times, which may particularly affect elderly individuals, patients with chronic illnesses, and those traveling long distances.

3.5 Workflow Coordination and Administrative Challenges

Shared patient experiences describe:

- Long waiting periods without clear communication
- Movement between multiple departments before reaching the correct clinic section
- Delays in consultation due to file retrieval processes

While such issues may arise from resource limitations and high patient volumes, record management practices appear to play a contributing role in overall workflow inefficiency.

3.6 Impact on Patients

The combined effect of these challenges may result in:

- Extended waiting times
- Increased travel and incidental costs
- Stress and inconvenience for patients and caregivers
- Reduced consultation efficiency

Although these observations do not represent formally quantified national statistics, the consistency of recurring themes across independent discussions suggests the presence of systemic administrative and documentation-related inefficiencies.

4. Preliminary Survey Findings

A structured questionnaire was distributed to nursing staff in selected government hospital clinics to collect primary data regarding record management practices.

Preliminary findings indicate:

- A significant proportion of respondents reported that retrieving paper-based records requires several minutes per patient.
- Respondents acknowledged occasional instances of incomplete or missing documentation.
- Many participants expressed that a digital system could improve efficiency and reduce administrative workload.

Data collection is ongoing, and additional responses will be incorporated into the final project documentation.

5. Limitations of Evidence

The qualitative observations presented in this document are based on publicly shared discussions, informal patient experiences, and limited preliminary survey data. These sources do not constitute peer-reviewed scientific research and therefore should not be interpreted as statistically representative of all government hospital clinics in Sri Lanka.

However, the consistency of recurring themes across independent discussions and survey responses suggests the presence of practical challenges related to paper-based record management.

Further quantitative data collection, including expanded patient surveys, is planned to strengthen the empirical foundation of the proposed solution.

6. Justification for the Proposed Solution

The recurring challenges identified in this document highlight several key concerns:

- Risk of document loss or damage
- Administrative inefficiencies
- Limited patient access to personal medical information
- Increased workload for healthcare staff
- Dependence on physical storage systems.

The proposed ClinicOne system aims to address these issues through:

- Secure digital storage of medical records
- Structured and searchable patient information
- Role-based access control
- Reduced dependence on physical documents
- Improved clinic workflow efficiency

By transitioning from paper-based record management to a secure web-based system, healthcare institutions may improve service quality, accessibility, and administrative coordination while maintaining data confidentiality and ethical standards.

7. Conclusion

Although formally published research on this specific issue within Sri Lanka remains limited, the qualitative evidence and preliminary survey findings presented in this document demonstrate recurring challenges associated with paper-based medical record systems in government hospital clinics.

These findings provide contextual justification for the development of ClinicOne as a structured digital clinical information management system designed to improve efficiency, accessibility, and healthcare service delivery.

Appendix A

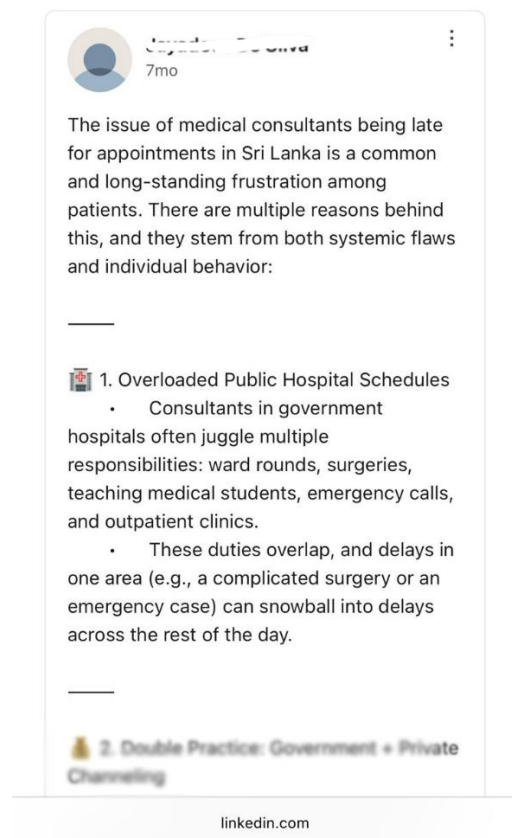


Figure 1 commentary-on-LinkedIn-1

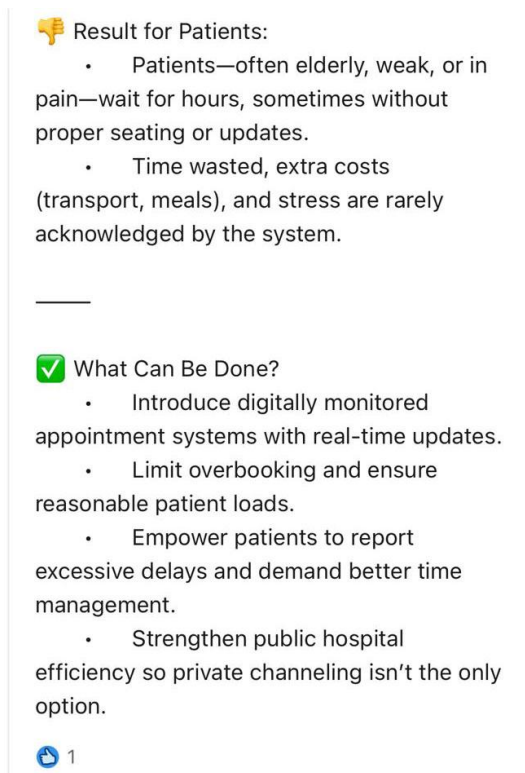


Figure 2 commentary-on-LinkedIn-2

Appendix B

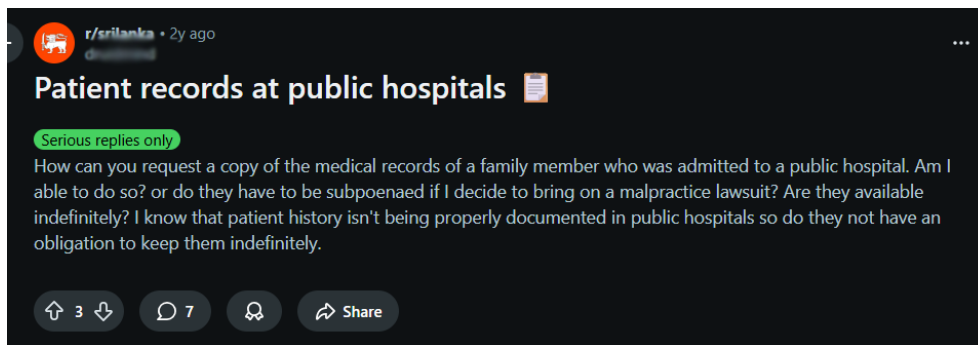


Figure 3 commentary-on-Reddit-1

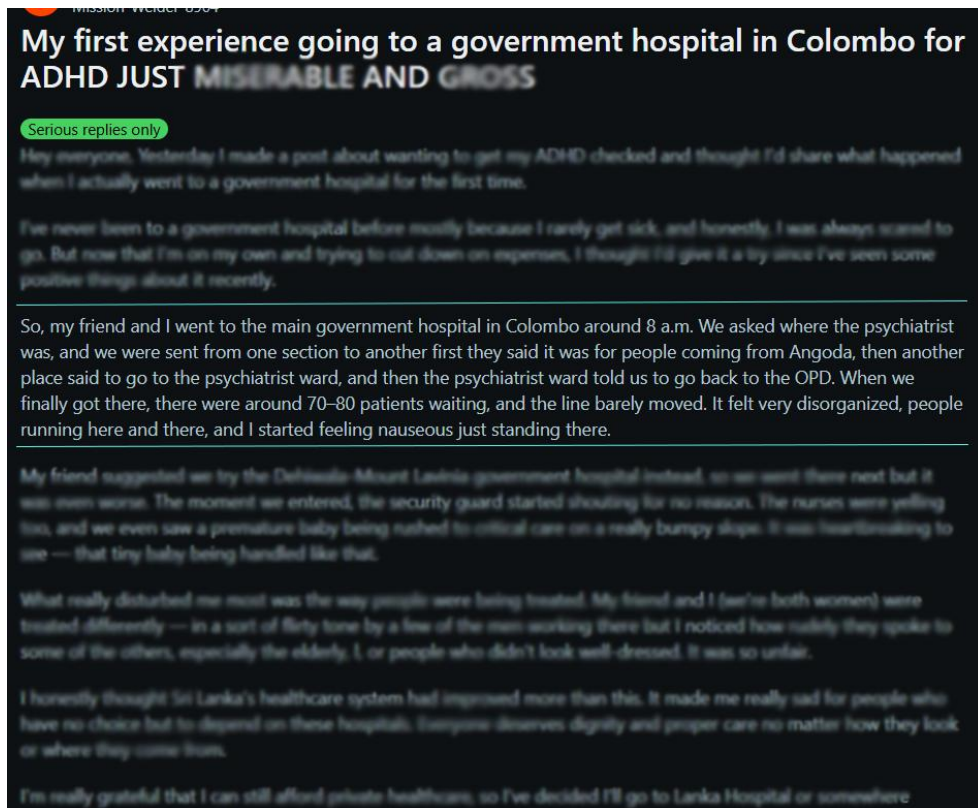


Figure 4 commentary-on-Reddit-2

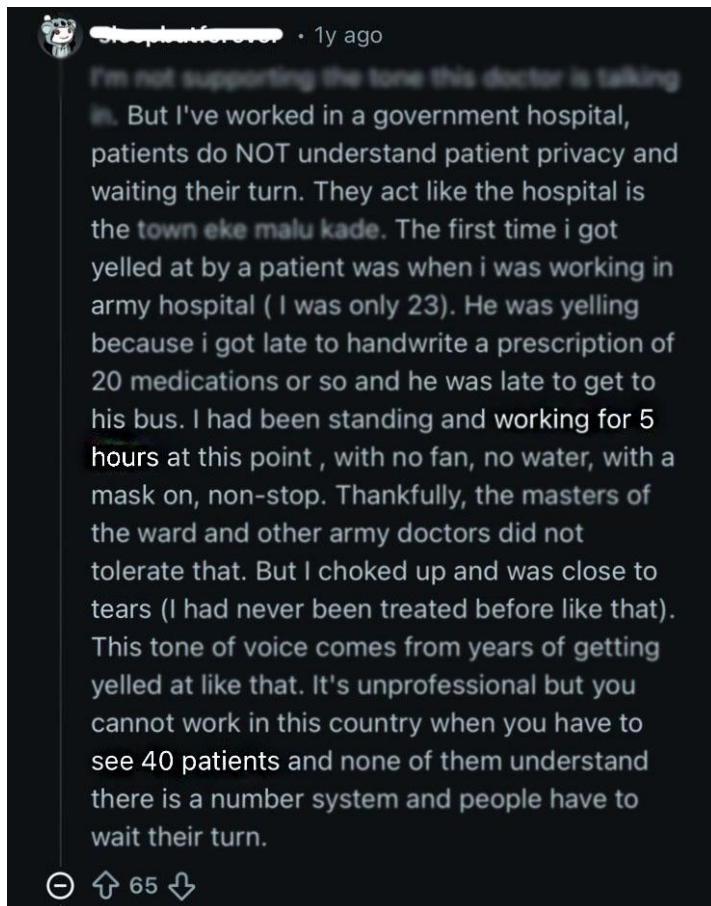


Figure 5 commentary-on-Reddit-3